



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : هند سيد عبدالعزيز محمد
المحافظة : القاهرة الحي / المدينة : الزيتون
رقم البطاقة : 28209270102087 رقم التليفون : 01141411874
العنوان : القاهرة حلوان
العمر (تاريخ الميلاد) : 27-09-1982 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 04-10-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: -----
الاسم: د/ندى حسن الوظيفة: -----
الاسم: د/نرمين رمضان الوظيفة: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /----- /أ.د. /----- /د. /----- /د. /-----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : قسم : -----
جهة العلاج : نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: هند سيد عبدالعزيز محمد National ID: 28209270102087	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





Patient Name	: الأستاذة / هند سيد عبد العزيز محمد	Request No.	: 13009837
Patient No.	: 28209270102087	Request Date	: 30/12/2024
Path No.	: 5251/24	Verification Date	: 28/01/2025
Sex	: Female	Financial Category	: أورام دار السلام - مبادره
Age	: 42 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Left breast malignant looking mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Equivocal (score 2+).
- Ki67: Positive in 17% of tumor cells.
- SISH: Amplified.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Prof. Dr.Amal Hareedy.

Dr.Rana Fathy.

Verified by :

Dr.Toka Medhat



Name: هند سيد عبدالعزيز محمد		
MR. : 1945		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient has metastatic breast cancer with bone involvement and is receiving HERCEPTIN IV and ZOMETA in addition to FEMARA; an echocardiogram performed on 3/2 showed an ejection fraction of 64%, and a new CT scan of the chest, abdomen, and pelvis was pending. The patient continued on FEMARA alone on the day of assessment, with HERCEPTIN IV and a repeat echocardiogram scheduled; a new CT scan of the chest, abdomen, and pelvis was still pending. The patient received HERCEPTIN IV, FEMARA, and ZOLADEX today, with the next ZOLADEX dose planned for 16-08; an echocardiogram will be performed at the next visit. Clinical pharmacy intervention documented that the patient had received HERCEPTIN and FEMARA the previous week and received ZOMETA today. A new echocardiogram was requested, and a bone scan was ordered; the patient continues on HERCEPTIN IV and FEMARA. The patient with metastatic breast cancer to bone is on paracetamol 1000 mg every 6 hours, and tramadol 50 mg every 12 hours was added for pain control. The patient remains on FEMARA and HERCEPTIN for metastatic breast cancer to bone; the most recent bone scan compared with April 2026 shows stable disease, and the CT CAP was free of progression, allowing continuation of the current treatment regimen.		
تحريراً في ,		
	الوقت :	التاريخ : 18-09-2026
هنا أسامه عبد الرحيم عبد البر		الطبيب المعالج :
يعتمد ,		



Patient Name	: الأستاذة / هند سيد عبد العزيز محمد	Request No.	: 13009837
Patient No.	: 28209270102087	Request Date	: 30/12/2024
Path No.	: 5251/24	Verification Date	: 28/01/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 42 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast malignant looking mass.

Nature of Specimen: Tru-cut biopsy, collected at 29/12/2024.

Gross Examination:

Four grayish white tissue cores up to 1 cm, totally submitted.

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2,SISH, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Undetected.
- * Perineural invasion: Undetected.
- * Angioinvasion: Undetected.
- * Microcalcifications: Undetected.
- * IPT (Positive control showed appropriate reactivity):
 - ER: Strong nuclear reaction in 70% of tumor cells (score 8/8).
 - PR: Strong nuclear reaction in 40% of tumor cells (score 7/8).
 - Her2: Moderate complete membrane reaction in >10% of tumor cells (score 2+).
 - Ki67: 17% nuclear reaction of tumor cells.
 - SISH: Amplified.

(Average Her2 copy No. (6.2), Her2/CEP17 ratio: (3.3))

Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.

Diagnosis:

Verified by :

Dr.Toka Medhat

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name	: الأستاذة / هند سيد عبد العزيز محمد	Request No.	: 13009837
Patient No.	: 28209270102087	Request Date	: 30/12/2024
Path No.	: 5251/24	Verification Date	: 28/01/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 42 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Dr.Samia Ali.

Verified by :
Dr.Toka Medhat

Printing Date : 28/01/2025 12:58:38

Page 3 of 3

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name: Mrs. HEND SAED ABDELAZIZ
Date: 10-12-2025

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: left breast cancer, underwent NACT , follow ups study.

Findings:

Right breast.

- Heterogenously dense breast parenchyma that may obscure small lesions. (ACR c)
- No speculations, masses asymmetry suspicious microcalcifications.
- UOQ focal asymmetry , likely at site of previous surgical drainage
- Benign calcifications
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Heterogenously dense breast parenchyma that may obscure small lesions. (ACR c)
- UIQ ill-defined spiculated mass lesion.
- Evidence of benign macrocalcification.
- **Ultrasound** demonstrated scattered simple cysts.
- 11-12 o'clock, zone b, 9 mm from skin, ill- defined hypoechoic spiculated mass lesion, measures 15 x 10 mm, with peripheral color uptake on doppler study.
- 12 o'clock, zone a small hypoechoic circumscribed nodule, measures 6 x 3 mm, with no color uptake on doppler study.
- A small 11 o'clock, zone a, rounded lesion measures 3 x 2 mm, with slight echogenic contents, likely complicated cyst
- Normal skin/nipple/areola complex.
-No pathological axillary, infra or supra clavicular lymph nodes.

Conclusion: -

Right breast (BIRADS 2): Bening macrocalcifications, post-surgical drainage sequelae

Left breast (BIRADS 6): 11-12 o'clock suspicious mass lesion, with 12 o'clock indeterminate nodule and 11 o'clock? complicated cyst.

Best Regards,
DR. RANIA AHMED, MD

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

<u>Category & recommendations</u>	<u>Suspected Cancer Risk</u>
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

PATIENT NAME: HEND SAYED ABDEL AZIZ MOHAMMED

GENDER: Female

DATE : 15-Apr-2026 11:49 AM

PATIENT ID: 10125259772

AGE: 43 y

ORDER: 3829337

99MTC-MDP BONE SCAN**Desired Information: -**
skeletal survey.**Technique:**

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report: Compared to previous bone scan dated 2/3/2025, the current study revealed :-

More sclerosis of the still seen suspicious DV10 lesion.

Degenerative changes are seen over dorso-lumbar vertebrae.

Arthritic changes are seen over both knees.

Both kidneys are normally visualized.

Conclusion:

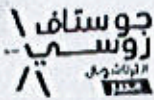
More or less stationary course of the DV10 lesion.

Reported By,

Prof. Huda Fathy, Nuclear Medicine Consultant

15-Apr-2026 12:31 PM





Patient's Name:

HIND SAYED ABDEL AZIZ

Date:

2026-04-04

Referred by:

Prof.Dr. CAP+C

Hospital:

Goustave Roussey International Egypt

Opinion:

❖ D10 vertebral body sclerotic area and left iliac bone tiny lytic focus are seen for bone scan.

Dr.sherif Wagdy

Dr.sherif Wagdy

Patient's Name: HIND SAYED ABDEL AZIZ
Date: 2026-04-04
Referred by: Prof.Dr. CAP+C
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

Known case of breast CA with no available previous films for comparison.

I- CHEST:

- Left basal atelectatic band.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Gall bladder shows a 21 mm stone.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- IUCD is seen within the uterus.
- Normal appearance of the rest of pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- D10 vertebral body predominantly sclerotic area is noted.
- Tiny left iliac bone lytic focus is seen.



Name: Hend Sayed Abd El-Aziz.

Date: 15.6.2025 .

Referred by Prof. DR. Mohammed Hefny.

MRN : 30192747.

F18-FDG PET/CT STUDY

CLINICAL HISTORY:

- * 42 y-old female patient of breast cancer.

REASON FOR PET/CT STUDY:

- * Assessment of response to therapy.

TECHNIQUE:

- Radiopharmaceutical: 18F FDG.
- Time from 18F-FDG injection to scan: 60 min
- PET/CT images were acquired from the skull base through the upper thighs & CT images were acquired for the purpose of anatomical localization and attenuation correction. Images were reviewed in the transaxial, coronal, and sagittal planes.
- Additional notes: SUV max normalized to body weight was used.

COMPARISON: No previous PET CT studies.



57357

مؤسسة مستشفى سرطان
الأطفال - مصر
Children's Cancer Hospital
Foundation - Egypt

Nuclear Medicine Dept.
PET/CT Unit

Musculoskeletal:

- * D10 shows mixed lytic/sclerotic osseous lesion of SUVmax~3.5.

IMPRESSION:

No definite metabolically active FDG avid gross breast lesions.
FDG avid D10 bone lesion.

Thanks for referral;
Nuclear Medicine Unit;

Dr. Mary Rabea , MD
Radiology consultant

Dr. Maha Mehesen, MD.
Nuclear medicine consultant.

1 Fostat street, El Sayda Zainab, Cairo
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رعاية علم، مقدمها الحياة
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الهاتف: (202) 19057
فاكس: (202) 25 35 1745



فند سيد عبدالعزيز محمد

DOB: 1962-09-27

Male

MRN: 289/2025

T FORM

24/12/2025 G

Name	فند سيد عبدالعزيز محمد	MRN	289/2025
Age	42	ID	CNC-9C-V-01-CAN
Menopausal status	Pre	Phone	011 41411 8701
NRS SCORE		Marital status	married
BMI	>30	Family history	
CO morbidity	no	REASON OF MDT	Surgery-

Clinical Data

BX:				R		L	
ER: 818	PR: 718	HER 2: +2 amplified	Ki67: 17%				
Sonomammography: by SISH							
BSM → 1+ breast masses							
Mets Workup: + axillary 1-N.							
CT CAP → HfIs → Triphasic → benign				TNM 3 1 1 (D10)			
Dio lesion → RIF							
Started Taxol / Herceptin 20mca.				34 weeks			
Recommendations: PET/CT 6RS → almost CR				last 12/11/2025			
PET/CT 25/11/2025 → CR				Continued Taxol / Herceptin			
BSM → Requested							
For MDT regarding surgical option							
if possible and Having AT / Zoladex							
MDT OUTCOME & PLAN							
Continue Herceptin 1-1 + Zoladex 3/12/2025.							
as pt not fit for stat surgery. No SBT plan cancelling							

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
M. Kadiy	Omar	No			
	24/12/2025	24/12/2025			